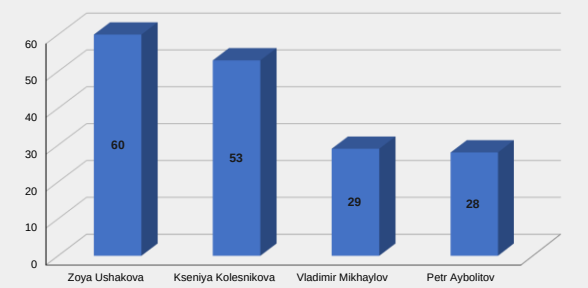
Analysis of the clinic's operating metrics over 15 months
to forecast results and recommend a strategy that maximises profit in quarter 5.

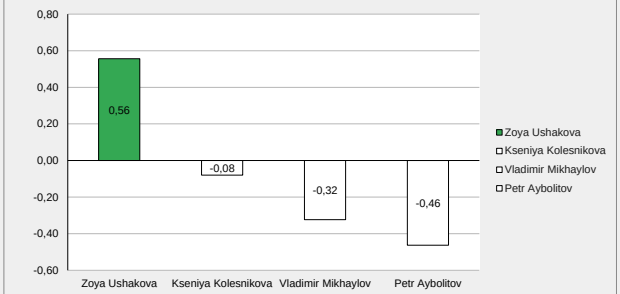
Clients acquired	Profit in Period 12	NPS Q1-Q4	Accumulated loss at end of Q4
3238	€ 3.382	Client loyalty increased	-€ 70.558,00
of 4,228 residents (77% know the clinic)	first profitable month	from -18% to 48%	

Hypothesis 1: Service quality and doctor workload.

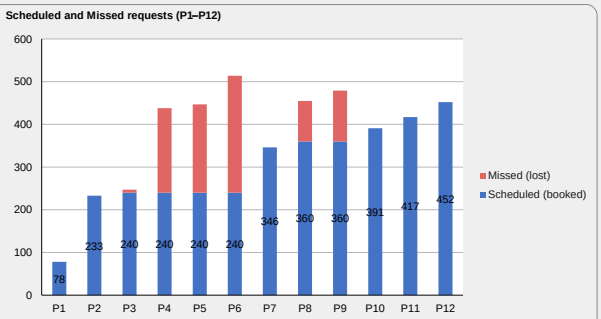
Do all doctors perform equally well, and does the clinic need a fifth specialist?



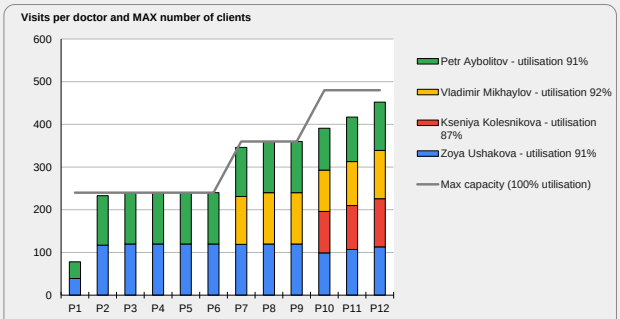
NPS (Net Promoter Score) for each doctor over all 12 periods.
Calculated as the difference between satisfied and dissatisfied clients divided by the total number of visits. Zoya Ushakova leads with an NPS of 60%. Petr Aybolitov and Vladimir Mikhaylov are in the risk zone at 28–29%.
The gap between the best and the worst doctor is more than twofold. Dissatisfied clients do not come back and do not recommend the clinic, which is a direct loss for the referral channel.



Average client rating of each doctor's service. This metric matters because satisfied clients are the ones who recommend the clinic to people they know. The data shows that advertising stopped in period 7 and only the referral program is running.
Only 1 doctor is positive at +0.56; all other doctors are negative.
Petr Aybolitov at −0.46 with 1,305 visits means thousands of clients left with a negative impression of the service. This is what holds back the growth of the referral base.



The chart shows, for each period, booked visit requests versus requests lost because no doctor was available. Blue: clients who were booked. Red: clients who wanted to book but could not.
In periods P4–P6 the clinic lost 198 to 274 clients a month. After the third doctor was hired in P7, losses dropped to zero. After the fourth was hired in P10 they stayed at zero and have not grown since.
This directly confirms that the current team of four doctors fully covers demand.

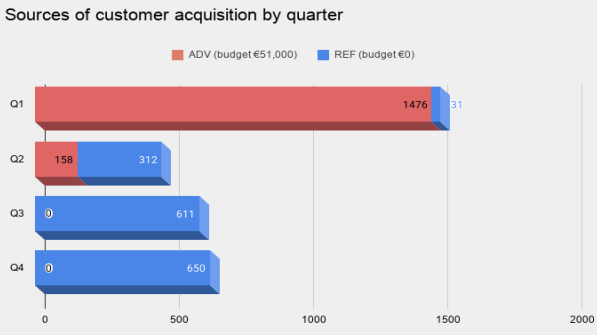


The chart shows each doctor's actual visits by period, plus a grey line for the clinic's maximum capacity. Capacity = number of doctors × 20 days × 6 appointments per day.
In periods P1–P6 the clinic worked with two doctors and a maximum of 240 visits. A third doctor was added in P7, raising capacity to 360.
A fourth doctor was added in P10, capacity 480. Doctor utilisation stays at a steady 87–92%, which is optimal. A fifth doctor is not needed.

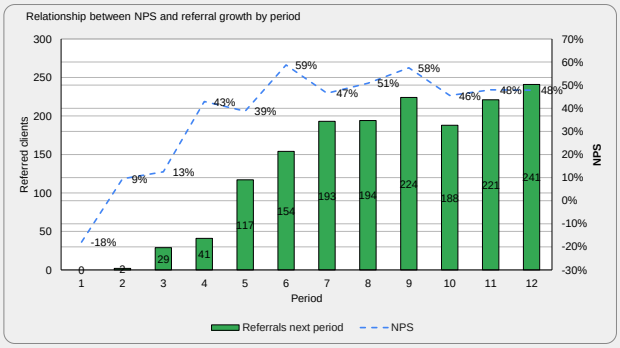
Conclusion: Hypothesis 1
Hypothesis confirmed. Doctors' work quality differs considerably: the NPS gap is more than twofold. At the same time clinic u tilisation is optimal and no clients are lost. Two recommendations: run customer-service training for Aybolitov and Mikhaylov, and do not hire a fifth doctor at this stage, as it would only add costs without adding revenue.

Hypothesis 2: Profit growth without new clients

How can the clinic grow revenue without adding staff or marketing? How can the referral program be improved?



New clients per quarter, stacked by channel: paid advertising (ADV) and referral program (REF).
It shows clearly how the channel mix changed over the year.
In Q1 almost all acquisition came from advertising: 1,476 clients for €45,000. By Q2 referrals were already catching up.
In Q3 and Q4 advertising was switched off completely, yet the clinic gained 611 and 650 new clients purely through referrals, at no cost. The inflow did not drop; it kept growing.



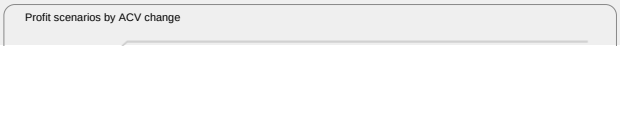
Combo chart: bars show the number of promoters (clients with PS ≥ 9) in each period; the line shows NPS.
There is a clear correlation: when NPS rises, the number of referrals rises in the next period.
This proves a direct link: service quality creates promoters, and promoters bring new clients for free. That is why investing in doctors' service quality pays off better than spending on advertising.

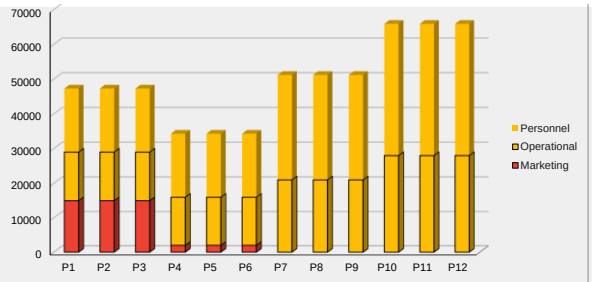
Conclusion: Hypothesis 2
Hypothesis confirmed. The referral channel now matches advertising in effectiveness at zero cost. Clients acquired are split almost evenly: 50.5% advertising vs 49.5% referrals, but the cost is fundamentally different. After advertising was switched off in Q3, the inflow of clients did not decrease. The referral program is fully se lf-sustaining at this stage.
What can work: maintain and grow the referral program by raising NPS. Every promoter with a score of 9 or higher brings in ne w clients for free. This is why improving doctors' service quality (Hypothesis 1) directly drives client base growth (Hypothesis 2).

Hypothesis 3: Q5 profit forecast

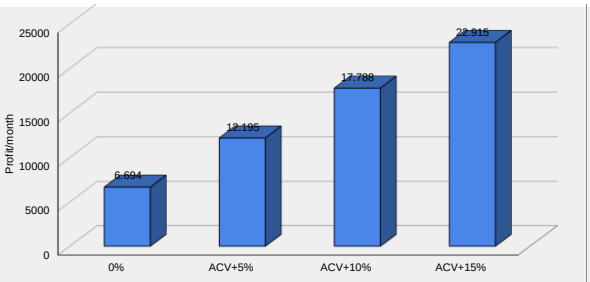
What increase in the average check would raise the clinic's profitability? How long will it take the clinic to cover its losses and become profitable?



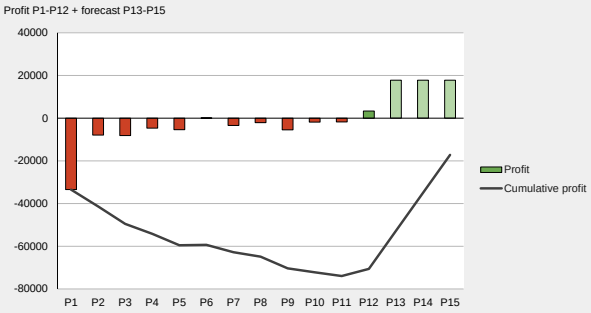




The largest cost item is doctor salaries: €37,900 (37% of all costs).
This is a fixed cost that does not depend on the number of clients seen: the clinic pays the same at 391 visits as at 480.
That is why utilisation is so critical: more clients at the same salaries means higher profit.
Operating expenses are also fixed: €28,000 (27%).
The only variable cost item is service costs: €34,426 (34%).
The more visits, the higher these costs. The ratio is 34.8% of revenue, so the clinic spends about 35 cents on materials for every euro earned.



We compare four scenarios for the average check: current level, +5%, +10% and +15%.
Y axis: expected monthly profit at 466 visits. The figures account for client growth over the last quarter.
Even with no changes, the clinic at full capacity earns about €6,694 per month. +5% adds another €5,498. +10% gives €17,788 per month. +15% gives the highest profit but risks losing price -sensitive clients. That is why I recommend the +10% scenario: it gives the best balance between profit growth and churn risk.



Combo chart: bars show profit or loss for each period; the line shows the cumulative total. Green bars: profit; light green: expected profit; red: loss. Periods P13–P15 are the forecast under the ACV +10% strategy.
For the first 11 periods the clinic operated at a loss. P12 was the first period with a solid profit of +€3,382.
With a 10% higher average check in Q5, profit grows to €17,788 per month. The accumulated deficit of −€70,558 starts shrinking fast: from P13 to P15 it falls by €53,363. Full payback is expected around period 16–17.

Conclusion: Hypothesis 3
The clinic can increase profit significantly without hiring new doctors and without marketing spend. The one lever is a 10% i ncrease in the average check. With the current client flow and a stable 90–94% utilisation, this is realistic and requires no additional investment.
Recommended Q5 strategy in three points:
Raise ACV by 10%, positioning it as a premium service rather than a simple price increase. The argument for clients is high -quality treatment and attentive service. Staff should also receive service-quality training.
This alone brings the clinic to a monthly profit of about €17,000 and fully pays off the accumulated deficit by around period 16–17.